

# Rib fracture management in adults

Version 10

For use as soon as diagnosis is clinically suspected

Not required if decision to transfer to the Major Trauma Centre (MTC) has already been made

**Disclaimer:**  
This is a clinical template; clinicians should always use judgment when managing individual patients

Re-approved by UHL POC on 17May24  
Review due Jul27 . Trust Ref: B27/2017

## Patient details

Full name

DoB

S number

(use addressograph if available)

## ① Analgesia bundle

- Paracetamol PO (unless taken within last 4h)
- Ibuprofen PO unless NSAID contraindicated
- Dihydrocodeine (elderly: codeine) 30mg PO
- If pain severe: Add morphine IV titrated
- **Consider** regional anaesthesia if suitable and expertise available (ED or anaesthetic team)
- **Consider** ketamine IV in adjuvant analgesic dose (up to 0.25mg/kg titrated) if pain recurs (to be given by ED consultant or registrar only)

## ② CT thorax indicated?

☐ **Yes**, as at least one of the below

- CT of other body regions required ☐
- Dangerous mechanism ☐
- New hypoxia ☐
- Tachypnoea ☐
- Haemodynamic instability ☐
- Multiple rib fractures on CXR ☐
- Lung contusions on CXR ☐

☐ **No**, as none of the above

## ③ Specialist care needed?

☐ **Yes**, as at least one of the below

- Flail chest ☐
- Bilateral rib # ☐
- Three or more rib # ☐
- Two or more rib # **AND** aged >65 ☐
- Chest drain required ☐
- Lung contusion ☐
- Persistent tachycardia ☐
- Persistent tachypnoea ☐
- Hypotension at any time ☐
- Fever ☐
- Significantly raised WCC ☐
- New oxygen requirement ☐
- Acute CO<sub>2</sub> retention ☐
- Chronic respiratory disease or heart failure ☐
- BMI 40 or greater ☐

☐ **No**, as none of the above

## ④ Critical care required?

☐ **Yes**, as at least one of the below

- Desaturating below target ☐
- SpO<sub>2</sub> despite high-flow O<sub>2</sub> ☐
- Acute CO<sub>2</sub> retention requiring NIV or invasive ventilation ☐

☐ **No**, as none of the above

## ⑤ Absolute indications for thoracic surgical care?

☐ **Yes**, as at least one of the below

- Chest drain required ☐
- Pain not controlled despite optimal initial analgesia **AND** ☐
- EITHER** >2 rib fractures ☐
- OR** aged >65 with >1 rib fracture ☐

☐ **No**, as none of the above

## ⑥ Discharge feasible?

☐ **YES**, as **ALL** of the below

- Age <65 ☐
- CFS <5 ☐
- Pain control adequate ☐
- Able to manage at home ☐
- No other indication for admission ☐

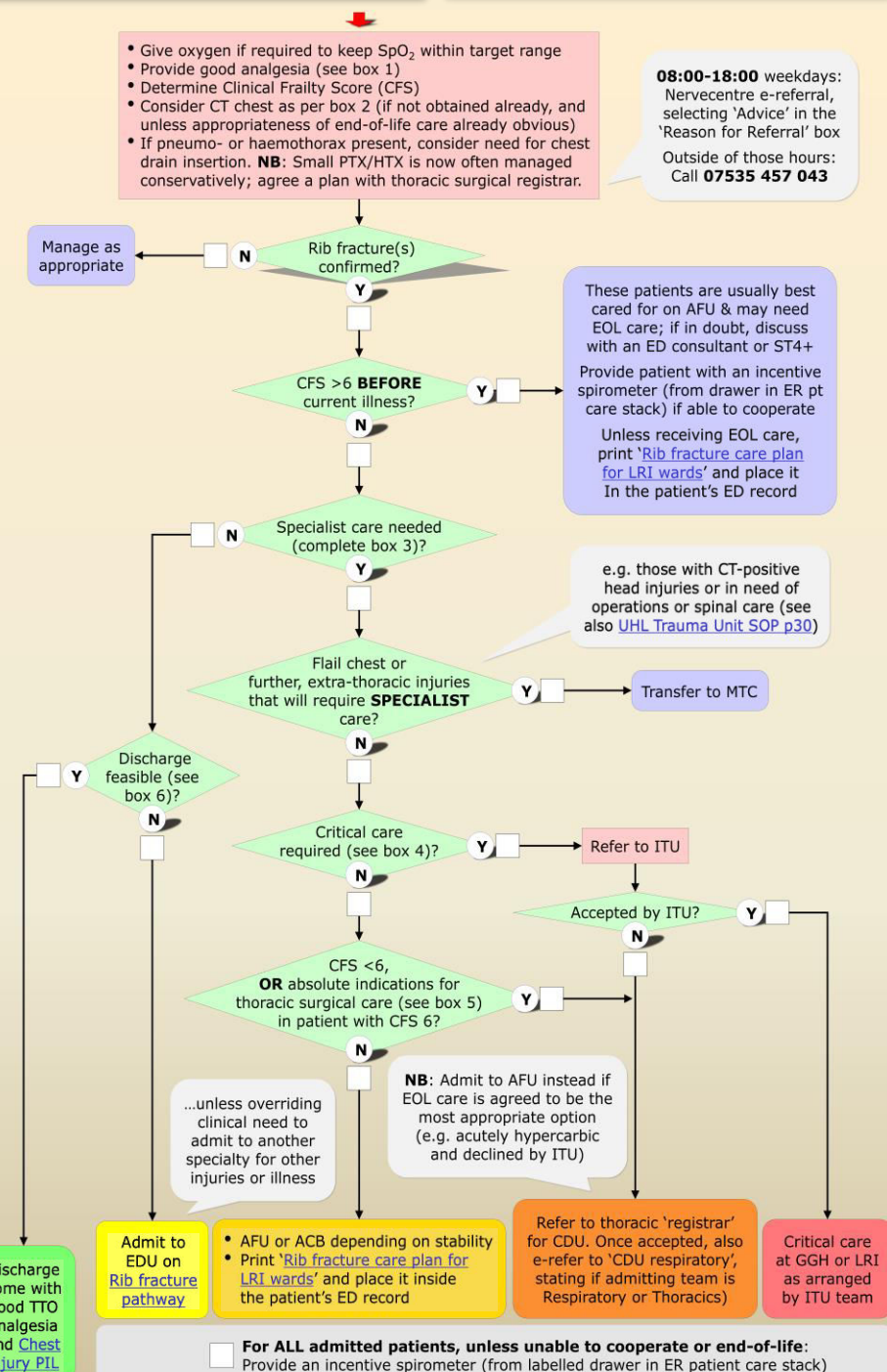
☐ **NO**, as not all of the above

- Give oxygen if required to keep SpO<sub>2</sub> within target range
- Provide good analgesia (see box 1)
- Determine Clinical Frailty Score (CFS)
- Consider CT chest as per box 2 (if not obtained already, and unless appropriateness of end-of-life care already obvious)
- If pneumo- or haemothorax present, consider need for chest drain insertion. **NB:** Small PTX/HTX is now often managed conservatively; agree a plan with thoracic surgical registrar.

**08:00-18:00** weekdays:  
Nervecentre e-referral, selecting 'Advice' in the 'Reason for Referral' box  
Outside of those hours:  
Call **07535 457 043**

These patients are usually best cared for on AFU & may need EOL care; if in doubt, discuss with an ED consultant or ST4+  
Provide patient with an incentive spirometer (from drawer in ER pt care stack) if able to cooperate  
Unless receiving EOL care, print 'Rib fracture care plan for LRI wards' and place it in the patient's ED record

e.g. those with CT-positive head injuries or in need of operations or spinal care (see also [UHL Trauma Unit SOP p30](#))



☐ **For ALL admitted patients, unless unable to cooperate or end-of-life:**  
Provide an incentive spirometer (from labelled drawer in ER patient care stack)

Managed by

Print name

Signature

Role

Date

Time